

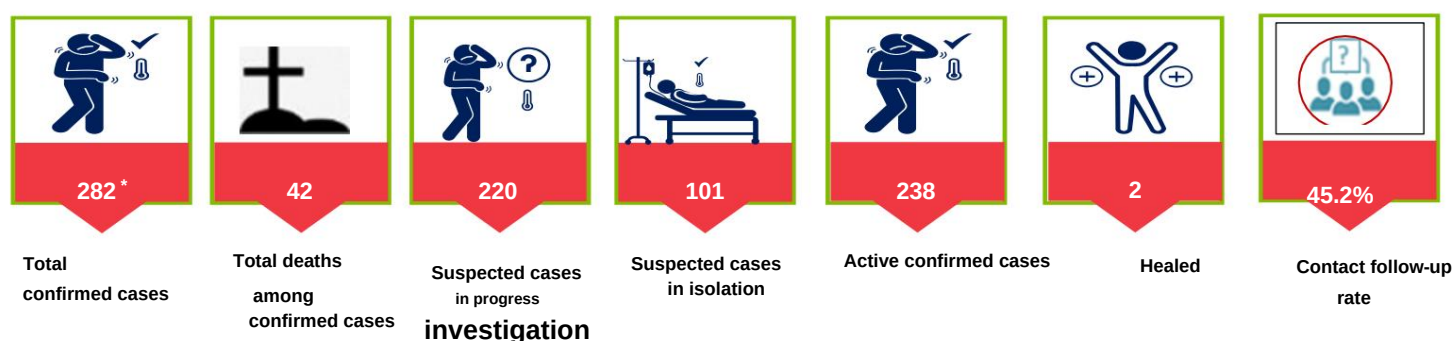
Public Health Emergency Operations Center
"COUSP DRC"

MVE-17 Incident Management System

2 Situation Report of the 17th Ebola Virus Disease Outbreak / DRC

SitRep N°016/MVB_30/2026

Provinces affected (3)	ITURI, NORTH KIVU & SOUTH KIVU
Affected Health Zones (22)	ITURI : Aru, Aungba, Bambu, Bunia, Damascus, Gethy, Kilo, Komanda, Lita, Mangala, Mongbwalu, Nizi, Nyankunde and Rwampara NORTH KIVU : Butembo, Goma, Oicha, Kalunguta, Katwa, Kyondo and Beni SOUTH KIVU : Miti-Murhesa
Reporting date	May 30, 2026
Publication date	May 31, 2026



*Data currently being harmonized

0. HIGHLIGHTS

- Arrival of the WHO Director-General and holding of the press conference with the Ministers of Health and Communication from the Congolese Government in Bunia;
- Official handover of 135 motorcycles, support from the central government for Ebola response activities in Ituri.
- No new provinces affected today.
- 19 new confirmed cases as of May 30, 2026.

I. CONTEXT

The Ituri province is located in the northeast of the Democratic Republic of Congo and shares a long border with Uganda and South Sudan. For more than two decades, it has faced a chronic humanitarian crisis linked to armed conflict and recurring population displacements. It has an estimated population of over 8 million, including more than one million internally displaced persons.

In September 2018, this province was affected by an Ebola virus disease epidemic that was raging in the neighbouring province of North Kivu.

The Mongbwalu health zone is one of the 36 health zones in Ituri province. Considered the starting point of the epidemic, it is located in the Djugu territory, where several armed groups operate. It is situated 70 km from Bunia, the capital of Ituri province, and experiences frequent population movements towards Uganda. This area is characterized by numerous construction sites that attract people from other territories within the province and from neighboring provinces.

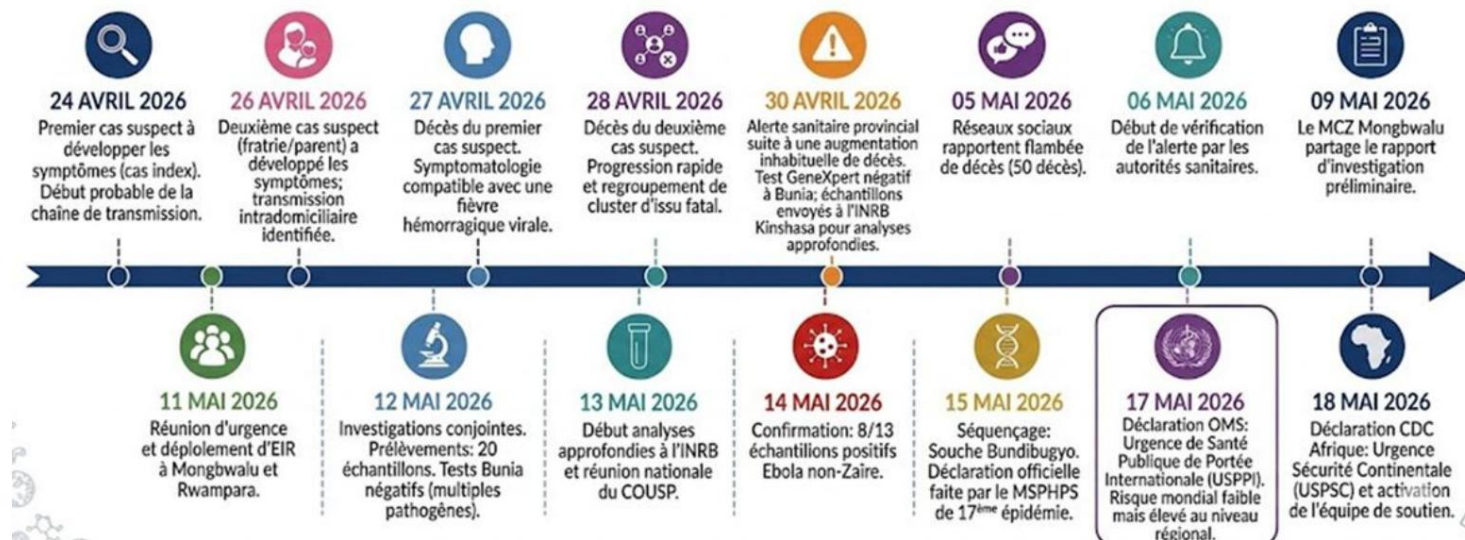
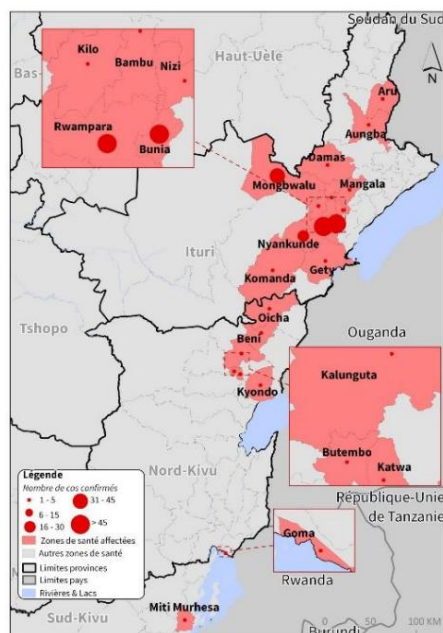


Figure 1. Chronology of events of the Ebola-Bundibugyo virus disease epidemic.

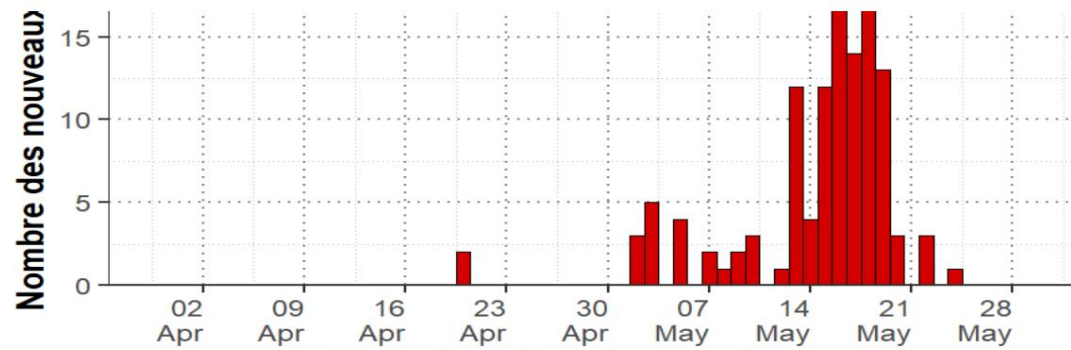


Figures 2 & 3. Spatial distribution of confirmed Ebola cases by affected health zone and in the three affected provinces as of May 30, 2026.

II. EPIDEMIOLOGICAL SITUATION UPDATE

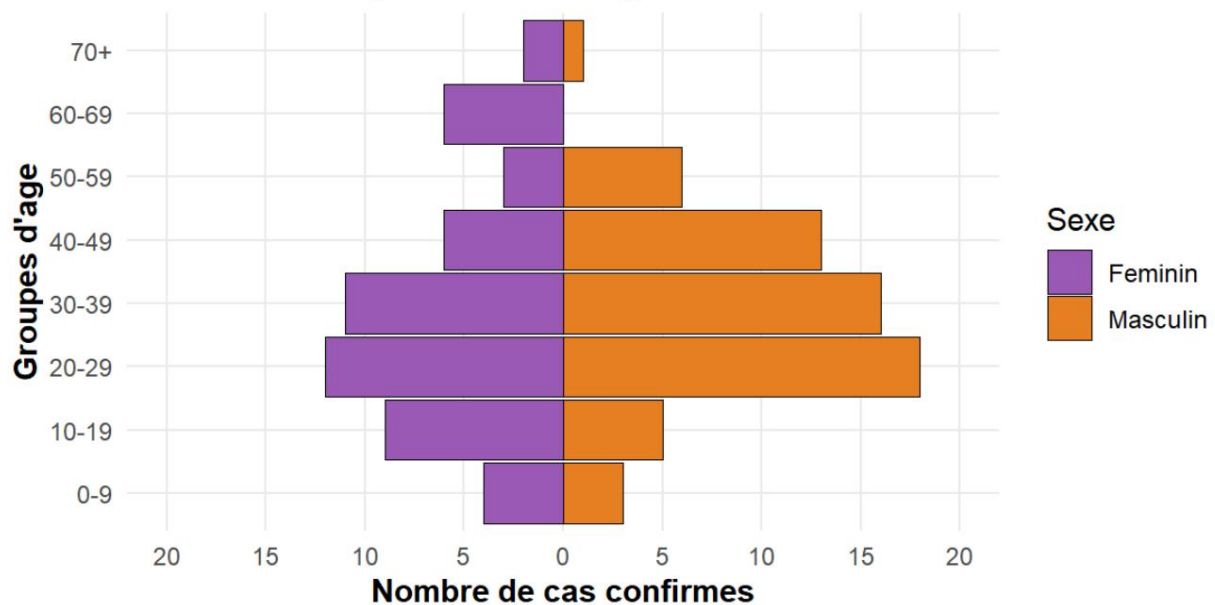
- 19 new confirmed cases were notified during the day on May 30, 2026.
- As of May 30, 2026, the cumulative number of confirmed cases stands at 282, including 264 in Ituri, 15 in North Kivu and 3 in South Kivu.
- Three provinces are currently affected by the epidemic: Ituri, North Kivu and South Kivu.
- The number of affected health zones remains at 14 in Ituri and at 22 nationally.
- The Ituri province remains the epicenter of the epidemic with 14 health zones affected out of 36: Aru, Aungba, Bambu, Damas, Bunia, Gety, Kilo, Komanda, Lita, Mangala, Mongbwalu, Nizi, Nyankunde and Rwampara.
- In North Kivu, 7 out of 35 health zones are affected: Goma, Beni, Butembo, Oicha, Kalunguta, Katwa and Kyondo.

Daily evolution of confirmed cases according to the date of onset of symptoms



After a few sporadic cases observed in late April and early May, a gradual and then rapid increase in confirmed cases of Ebola virus disease (EVD) was noted during May 2026, with a peak observed around May 17–18. A slight decrease in reports was observed at the end of the period, although transmission remained active.

Cas confirmés par tranche d'âge et sexe



acker - Riposte MVE, mai 2026. Données incluses dans l'analyse : n cas confirmés = 115.

The 20-49 age group is the most affected, with a predominance of males. The extreme age groups (0-9 years and ≥70 years) remain the least affected.

III. RESPONSE ACTIONS NSE

IV.1. COORDINATION

- Official handover of motorcycles to the Ituri Provincial Health Directorate (DPS) and the General Health Service (SGI) by the Minister of Health to support the activities of the riposte.
- Support for the laboratory pillar for data harmonization.

IV.2. EPIDEMIOLOGICAL SURVEILLANCE

- Reorganization of the pillar and deployment of epidemiologists in sub-pillars and health zones hotspots.
- Implementation of an early warning system at the ISP BUNIA displaced persons site together with the Bunia ZS.
- Routine field surveillance activities (reporting alerts, investigations, active searching, listing and monitoring contacts, retrospective documentary review of cases and deaths, etc.).
- Support for RECO's briefing on community-based surveillance and contact tracing in the ZS of Aru and Bunia.
- For May 29, 2026, 149 alerts were reported, of which 84 (56.3%) were investigated and 73 (86.59%) validated, of which 33 (45.02%) were subject to sampling.



Point of Entry and Point of Control (PoE/PoC) Surveillance:

- Development of the schedule of intensive activities to be carried out over twelve (12) days at the points of entry (PoE) the 11 activities.
- Data encoding and updating of the PoE/PoC database
- Updating of the population movement mapping of the Bunia and Rwampara health zones is underway.

Table I. Summary of PoE/PoC data as of 30/05/2026

Indicators at PoE/PoC	TOTAL
Travelers who have used the PoE/PoC	13,960
Screened travelers	13,187 (94.4%)
Travelers who have washed their hands	13,187 (94.4%)
Travelers made aware of Ebola virus disease	13,960 (100%)
Listed contacts	0
Alerts raised	0

IV.3. LABORATORY

- Analyses continued at the Bunia laboratory and the INRB Kinshasa. • At the end of the day on May 30, 2026, 52 new samples were collected and analyzed at the laboratory, 19 of which came back positive (36.5% positivity rate).

IV.4. INFECTION PREVENTION & CONTROL

- Official launch of PCI-WASH training for two hundred and eighty (280) healthcare providers from different structures (state, private and traditional practitioners) in the Mongbwalu Health Zone.

1. ZS Bunia

- Provision of emergency PPE to CH UNIBU and Clinique SOFEPADI.
- Two (2) EDS carried out on two (2) alerts raised as well as two (2) decontaminations one at the (HGR and another in AS Bigo).

2. ZS Rwampara

- Provision of PCI kits to BCZ Rwampara.
- Three (3) EDS (Emergency Response Systems) carried out on two (2) alerts reported: one to the HGR (General Referral Hospital), one (1) to the Balidja Morgues, and one (1) to the CME (Medical Emergency Center) Nyakunde) and three (3) household decontamination operations.
- Evaluation score card of CS Tchungu 24.2%.
- Briefing with the IT department of CS Tchungu and three (3) healthcare providers (CM Divine) on the importance of hygiene hand hygiene, injection safety and waste management.

3. ZS Aru

- Briefing of thirteen (13) CH Aru healthcare providers on the wearing of PPE and waste management with a scorecard assessment of 34%.

4. ZS Mongbwalu

- Start of briefing for 20 service providers, including a decontamination team for the 5 AS axis (Lodjo, AS Pili) Pili, AS Pluto, AS Yedi and AS Ngamau).
- Decontamination of a room at the Mongbwalu General Referral Hospital (internal medicine), 5 households including 3 at the NDCM Health Center and 2 others at AS CECA 20.
- 13 EDS carried out out of 15 alerts reported, including 8 to AS NDCM, 3 to AS CECA20, 1 to AS Mongbwalu, and 1 to AS Albekongo.
- In addition, a postponement of EDS Mongbwalu is not carried out due to refusal by the family (an EDS in the AS).
- Provision of 2 PCI Kits to CS Yedi and CS Ngamau.

IV.5. HOLISTIC CARE

- For the day of May 30, 2026, seven (7) deaths were reported and twelve patients are in critical condition.
- Joint PECH-PCI-Surveillance-Unicef descent (assessment of the Nyankunde SZ for MVE intensification);
- Monitoring of CTE construction work;
- Transfer of patients from ESS to PEC sites.

Psychosocial interventions and mental health

- Thirty-seven (37) results (reports) including twenty (20) positive ones (Nyankunde, Rwampara, Mungwalu, Boga, Lita and Bunia);
- Two (2) deaths (CTE Bunia and HGR Rwampara) • Thirty (30) suspected cases on sampling and isolation (CME); • With the staff of intensive care and maternity (Rwampara and Bunia);
- Two (2) families of the deceased to prevent complicated grief (Rwampara, CME)
- 6 PPL in isolation (CME, HGR Rwampara, Bambu and Nyankunde) • Six (6) military personnel are currently bedridden in the military pavilion and their families;
- Twelve (12) households and taxi drivers from the 6 parking lots near the facilities (HGR, CME and some CS)
- Fifteen (15) religious leaders, fifty-seven (57) students and thirty-three (33) teachers, 205 taxi drivers (Nyankunde, Rwampara, Aru, Bambu and Bunia);
- Twenty-one 21s to the staff (CME, HGR Rwampara, Bambu and Bunia)

Table II. Patient movement in healthcare facilities as of 30/05/2026

ISSUES		Buira	HGR Rwampara	CME	HGR Mongwalu	HGR Nyankunde	Buira	HGR	Total
Report (Number of patients in bed at the end of the previous day)		17	14	10	11	41	5	5	103
News Today's admissions	PPL	0	0	0	0	0	1	0	1
	Others	5	3	7	6	1	4	0	26
Outings	Healed	0	0	0	0	0	0	0	0
	Deceased	0	2	2	3	0	0	0	7
	No Case	0	0	0	0	0	0	0	0
	Escapees	1	0	0	2	2	0	0	5
	Transferred to HGR	0	0	0	0	0	0	0	0
Patients in isolation	Total	23	19	19	22	44	10	5	142
	NC confirmed	1	0	0	0	0	0	0	1
	AC confirmed	1	1	1	0	12	0	1	16
	Suspects	19	14	14	12	28	10	4	101
	Suspect Child < 5 years	1	0	0	0	0	0	0	1

S: Suspects C: Confirmed ND: Not available

IV.6. CREC

ZS Bunia

- Two hundred and fifty (250) members of the Adventist church were educated in Swahili on preventive measures against Ebola Virus Disease (EVD) and the reporting of disease alerts. This took place during a Swahili-language service at the Adventist Church.
- One thousand six hundred and thirty-six (1636) students, including nine hundred and eighty-six (990) girls, were sensitized in the EP Don de Dieu, Lycée Chem-Chem, CS le Sommet schools on preventive measures against Ebola virus disease (EVD) in schools and their respective homes.
- **Continuation of the RECOS briefing on Community-Based Surveillance with reporting of disease alerts in 5 different sites.**
Result: one thousand five hundred and nine (159) RECOS reached on day (2)

ZS Rwampara •

Forty-five (45) motorcycle taxi drivers were sensitized on preventive measures against Ebola virus disease in two parking lots (Fifteenth Brigade and Daniel Station);

- Handing over of fifty (50) posters to the MAKI MBUDHA school after raising awareness among students and teachers

ZS Bamboo

- Three hundred and five (305) students, including one hundred and two (102) girls, were sensitized at EP KILOMOTO regarding the measures preventive measures and the escalation of alerts.

ZS Aru

- Advocacy with the President of the Territorial Youth Council for his involvement in the activities CREC in support of the Ebola response

- A planning meeting was held within the One Health sub-pillar with national experts from CCUS and DLMA (Directorate of Animal Disease Control). **Result** : A One Health activity plan was developed.

ZS NYANKUNDE

- Seventy-eight (78) people, including twenty-five (25) women and fifty-three (53) men, were sensitized in the village of BAMBILI on preventive measures against Ebola, particularly proper handwashing and acceptance of the EDS in the BADIYA AS
- Training of twenty-five (25) RECOS on community-based surveillance

IV.7. LOGISTICS

- Delivery of 5 tonnes of UG-PDSS support medicines to the Bunia Health Zone.
- Delivery of barrier netting to the CME Bunia.
- Delivery of PCI inputs to the Provincial Laboratory.
- Twenty-seven (27) vehicles and (4) Ambulances were made available to the various pillars to ensure their mobility to the following axes: Rwampara, Nyakunde, Bunia, Nizi, Bambu, Mongbwalu, Ariwara.
- Continued delivery of motorcycles (Unicef) to the ZS of Rwampara twenty-one (21) Mangala two (2) Biringi (1) Linga (1) Rethy (2) Lolwa (1).
- Monitoring of the construction work of the CTE at the HGR Bunia, at the CME Bunia, at the HGR Rwampara.

IV.8. SECURITY

- The security situation in the city of Bunia (Bunia and Rwampara health zones) is relatively calm.

IV. CHALLENGES

- Reluctance towards post-mortem sampling: Formal refusal by families to have diagnostic tests (swabs) carried out on the bodies of the deceased in morgues.
- Absence of standardized Ebola Transit Centre and Treatment Centre in Ituri.
- Weakness in Contact Follow-up and Lack of Training for Stakeholders
- Persistent gaps in the identification and rigorous monitoring of contact cases at the level of active and high-risk health zones.
- Shortages in isolation facilities: Total absence of essential medications at the Center Ebola treatment (ETC) and at the isolation block in Rwampara and Nyankunde.
- Supply failure of reference hospitals: Lack of supply of Essential Generic Medicines (EGMs) within the four targeted General Reference Hospitals (GRHs), weakening the overall care of patients.
- Lack of PCI equipment (PPE, DLM, chlorine) in North Kivu
- Low reporting of alerts by zones in the 3 affected provinces (Ituri, North Kivu and South Kivu) Kivu);
- Insufficient financial resources to support all pillars of the response in the health zones affected;
- Difficulty in transporting samples to laboratories in Goma and Kinshasa.
- Spread of rumors and proliferation of traditional treatment recipes.
- Lack of communication materials and tools.

V. PERSPECTIVES

- Training of service providers on the use of PCR RADIONE.
- Training of service providers on Ebola virus disease (EVD) and squamous cell carcinoma (SCC) surveillance in affected and high-risk health zones risk of three provinces being affected.
- Training of community health workers on Ebola virus disease and SBC surveillance in affected health zones and high risk in the province of ITURI.
- Finalize the development of the research protocol and the data collection tools for perception communities on Ebola, the response and actors in health zones affected by the Ebola epidemic.

The effective delivery of 1800 motorcycles has begun, with support from the central government.



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